

Dynamic Guideline: WHO-Based Pneumonia - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Evidence-status statement

As of **29 April 2026**, the most current WHO pneumonia-management guideline identified in the provided evidence is **WHO, Guideline on management of pneumonia and diarrhoea in children up to 10 years of age**, listed by WHO as a latest pneumonia publication dated **31 December 2024** [WHO Pneumonia](#). The guideline covers **clinical assessment, investigation, diagnosis, treatment and follow-up** for children with pneumonia or diarrhoea up to **10 years of age**, across **community, first-level health facility and hospital** care [WHO guideline PDF](#).

A WHO evidence brief titled **“Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)”** was identified, but it is **not a pneumonia guideline**; it concerns the WHO abortion care guideline and was launched through a WHO webinar scheduled for **11 December 2025** [WHO webinar/event page](#). No direct WHO source in the provided evidence confirms a pneumonia guideline titled **“Meaningful youth engagement in a WHO guideline development process: case study of WHO [] guideline”** or a WHO pneumonia guideline published on **29 April 2026**.

1.2 Exactly 5 WHO Guidelines or WHO Guideline-Linked Technical Documents Synthesized

No.	WHO source	Publication / update date identified	Population	Pneumonia scope	Role in this report
1	WHO. Guideline on management of pneumonia and diarrhoea in children up to 10 years of age	31 December 2024	Children up to 10 years	Clinical assessment, investigation, diagnosis, treatment and follow-up across community, first-level facility and hospital care	Primary/latest WHO pneumonia-management guideline WHO Pneumonia , WHO guideline PDF
2	WHO. Revised WHO classification and treatment of childhood pneumonia at health facilities	2014	Children 2–59 months	Simplified childhood pneumonia classification and treatment; oral amoxicillin replaces cotrimoxazole	Key prior WHO classification and antibiotic guidance WHO publication page , WHO evidence summary PDF
3	WHO/UNICEF IMCI chart booklet	Date not specified in provided summary	Children assessed under IMCI	Cough/difficult breathing classification, pulse oximetry if available, referral threshold, amoxicillin and follow-up	Operational primary-care algorithm WHO IMCI chart booklet PDF

No.	WHO source	Publication / update date identified	Population	Pneumonia scope	Role in this report
4	WHO. Recommendations for management of diarrhoea and pneumonia in HIV-infected and HIV-exposed infants and children	Date not specified in provided summary	HIV-infected and HIV-exposed infants and children	Severe/very severe pneumonia antibiotic regimens, treatment failure, GRADE methodology	Special-population pneumonia guidance WHO recommendations PDF
5	WHO. Hospital care for children / pocket-book hospital-care guidance	Date not specified in provided summary	Severely ill children in low-resource referral hospitals	Emergency airway/breathing management, oxygen, inpatient care for major childhood mortality causes including pneumonia	Hospital supportive-care and emergency-management guidance Hospital care for children – IRIS

1.3 Primary WHO Guideline: Overview

Item	Summary
Guideline title	Guideline on management of pneumonia and diarrhoea in children up to 10 years of age
Publication date	31 December 2024 on WHO pneumonia health-topic page WHO Pneumonia
Target population	Children with pneumonia or diarrhoea up to 10 years of age
Care settings	Community, first-level health facilities and hospitals WHO guideline PDF
Scope	Clinical assessment, investigation, diagnosis, treatment and follow-up
Exclusions	Preventive interventions, because WHO immunization, nutrition and environmental-health departments lead those areas WHO guideline PDF
Goal	Develop, update and consolidate recommendations so countries can revise or develop clinical protocols for children up to 10 years of age WHO guideline PDF

1.4 WHO Key Recommendations for Pneumonia

Table 1. WHO-Based Pneumonia Recommendations With GRADE Ratings

Clinical area	Recommendation	Target population	GRADE evidence quality	Strength
Non-severe pneumonia:	Treat with oral amoxicillin for 3 or 5 days when pneumonia is characterized only by fast	Children 2–59 months	Moderate	Strong WHO guideline PDF

Clinical area	Recommendation	Target population	GRADE evidence quality	Strength
fast breathing only	breathing, with no chest indrawing and no general danger signs			
Non-severe pneumonia with wheeze and no fever	Do not routinely use antibiotics , because viral illness is more likely	Children 2–59 months	Low	Strong Child Health – IRIS
Non-severe pneumonia without wheeze	Treat with oral amoxicillin ; in low-HIV-prevalence settings, use at least 40 mg/kg per dose twice daily for 3 days	Children 2–59 months	Moderate for amoxicillin effectiveness, based on WHO-linked evidence	Strong Child Health – IRIS
Pneumonia with chest indrawing, no danger signs	Oral amoxicillin can be used in selected outpatient children under revised WHO classification, replacing cotrimoxazole as first-line therapy	Children 2–59 months	Moderate	Strong WHO publication page , WHO/UNICEF quick reference guide
Severe pneumonia or very severe disease	Use injectable/parenteral antibiotics and urgent referral or hospital management; prior WHO-linked sources describe ampicillin or penicillin plus gentamicin , with ceftriaxone if not improving	Children with danger signs or severe disease	Low to Moderate , depending on population and regimen	Strong or Conditional , depending on subgroup NCBI Bookshelf / WHO recommendations , WHO recommendations PDF
HIV-exposed or HIV-infected severe/very severe pneumonia	Use ampicillin or penicillin plus gentamicin , or ceftriaxone , as first-line treatment	HIV-exposed or HIV-infected infants and children under 5 years	Low	Conditional WHO recommendations PDF
Treatment failure after first-line amoxicillin	Refer to a facility where appropriate second-line treatment is available	Children with non-severe pneumonia failing first-line therapy	Very Low / expert opinion	Conditional / weak Child Health – IRIS
Hypoxaemia / oxygen	Use pulse oximetry if available; refer if oxygen saturation is <90% ; provide oxygen for airway or breathing emergencies	Children with cough/difficult breathing or emergency signs	Low to Moderate	Strong , because benefit outweighs risk
Follow-up	Reassess in 3 days ; urgently refer if danger signs, stridor,	Children treated outpatient for	Low / operational	Strong WHO IMCI chart booklet PDF

Clinical area	Recommendation	Target population	GRADE evidence quality	Strength
	worsening chest indrawing, worsening respiratory rate, persistent/worsening fever or poor feeding; complete antibiotics if improving	pneumonia	evidence and expert consensus	

1.5 Diagnosis, Classification and Initial Assessment

Practical WHO-based classification

Clinical presentation	WHO-based interpretation	Recommended action
Cough or difficult breathing plus fast breathing only , no chest indrawing, no danger signs	Non-severe pneumonia	Oral amoxicillin for 3 or 5 days in children 2–59 months WHO guideline PDF
Cough or difficult breathing plus wheeze but no fever	Likely viral wheezing illness rather than bacterial pneumonia	Antibiotics not routine Child Health – IRIS
Cough or difficult breathing plus chest indrawing , without danger signs	Pneumonia under simplified WHO 2014 classification; outpatient oral amoxicillin may be appropriate in selected children	Oral amoxicillin and close follow-up WHO publication page
Any general danger sign, stridor in calm child, cyanosis, grunting, inability to drink/breastfeed, lethargy/unconsciousness, convulsions or vomiting everything	Severe pneumonia / very severe disease	Urgent referral, injectable/parenteral antibiotics, oxygen if hypoxaemic or respiratory emergency NCBI Bookshelf / WHO recommendations
SpO ₂ < 90% if pulse oximetry is available	Hypoxaemia	Refer and provide oxygen WHO IMCI chart booklet PDF

1.6 Treatment and Management Pathway

1. Assess severity immediately

- Check for general danger signs, respiratory distress, chest indrawing, stridor, cyanosis, grunting and ability to drink or breastfeed.
- Use pulse oximetry if available; **SpO₂ <90%** should trigger referral and oxygen support [WHO IMCI chart booklet PDF](#).

2. Treat non-severe bacterial pneumonia

- For children **2–59 months** with fast-breathing pneumonia only: **oral amoxicillin for 3 or 5 days** [WHO guideline PDF](#).
- Earlier WHO materials support amoxicillin as preferred first-line therapy and note replacement of cotrimoxazole [WHO publication page](#).

3. Avoid antibiotics when bacterial pneumonia is unlikely

- In children 2–59 months with non-severe pneumonia, **wheeze and no fever**, antibiotics are not routinely recommended [Child Health – IRIS](#).

4. **Escalate severe disease**

- Severe or very severe pneumonia requires urgent referral or hospital care, parenteral antibiotics, oxygen when hypoxaemia or respiratory emergency is present, and reassessment within 48–72 hours if not improving [NCBI Bookshelf / WHO recommendations](#), [Hospital care for children – IRIS](#).

5. **Follow up**

- Reassess outpatient children after **3 days**.
- Refer urgently if danger signs, stridor, worsening respiratory status, persistent/worsening fever or poor feeding occur.
- Complete antibiotics if breathing is slower, chest indrawing is absent, fever is less and feeding is improved [WHO IMCI chart booklet PDF](#).

1.7 Guideline Development Methodology

Methodological domain	WHO-based summary
Evidence review process	The 2024 WHO guideline was developed to address evidence emerging since earlier WHO pneumonia and diarrhoea guidance, including WHO-supported research and scoping reviews in 2020–2021 on etiology, diagnosis, treatment and follow-up WHO guideline PDF .
Guideline quality assurance	WHO defines guidelines as recommendation-containing products for clinical practice or public-health policy, and states that the Guidelines Review Committee ensures methodological quality and transparent evidence-based decision-making WHO Guidelines .
GRADE approach	WHO-linked HIV-exposed/infected pneumonia recommendations explicitly describe use of the GRADE approach, considering evidence quality, benefits and harms, values, preferences, costs and feasibility; recommendation strength was graded as strong or conditional WHO recommendations PDF .
Consensus method	WHO guideline-development groups provide technical and normative advice; where evidence is limited, recommendations may be based on expert consensus WHO recommendations PDF , WHO PDF on azithromycin MDA .
Conflict of interest management	WHO states that participating experts, disclosed interests and mitigation measures are publicly reported according to WHO policies WHO PDF on azithromycin MDA .
Youth engagement	WHO has a youth-engagement initiative and an evidence brief on meaningful youth engagement in guideline development, but the identified case study is the 2022 abortion care guideline , not pneumonia WHO Youth Engagement , WHO webinar/event page .

II. Evidence Summary After WHO Latest Guideline Release (Guideline on management of pneumonia and diarrhoea in children up to 10 years of age) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search strategy

Evidence considered after the WHO latest guideline release of **31 December 2024** through **29 April 2026** included:

- WHO official documents and WHO technical pages.
- Major non-WHO guidelines and updates: ATS, IDSA/PIDS, NICE and BMJ Best Practice.
- Systematic reviews, meta-analyses and RCT-informed summaries.
- Evidence on antibiotic duration, corticosteroids, rapid diagnostics, site-of-care decisions and pediatric complicated pneumonia.

Databases and sources represented in the provided evidence

Source category	Examples used
WHO official sources	WHO pneumonia page, WHO guideline PDF, WHO guidelines page, WHO youth engagement page
Pediatric CAP guideline updates	IDSA/PIDS 2026 pediatric CAP guideline page IDSA
Adult CAP guideline updates	ATS 2025 adult CAP update ATS
Evidence summaries	BMJ Best Practice adult and pediatric CAP summaries BMJ Best Practice PDF , BMJ Best Practice: pediatric CAP
NICE evidence reviews	NICE 2025 pneumonia diagnosis/management and corticosteroid evidence reviews NICE PDF
Reviews and RCT-informed evidence	Recent diagnosis/treatment review with rapid diagnostics Pneumonia: Recent Updates on Diagnosis and Treatment ; antibiotic-duration evidence summarized by ATS and Stanford pathway Contagion Live , Stanford SHC pathway PDF

Inclusion criteria

- Published or updated after **31 December 2024**, or older high-quality evidence directly used in 2025–2026 updates.
- Pneumonia diagnosis, treatment or management.
- Pediatric or adult CAP, complicated pneumonia, HAP or VAP evidence where clinically relevant.
- Sources reporting GRADE or allowing report-level GRADE assessment.

Exclusion criteria

- Non-pneumonia respiratory infections without pneumonia-specific recommendations.
- Prevention-only evidence unless directly relevant to management.
- Non-guideline opinion pieces without evidence synthesis.
- Sources not represented in the provided summaries.

2.2 Key New Studies and Guideline Updates

Table 2. Post-WHO-Release Evidence With GRADE Ratings

Evidence / source	Design	Population	Key findings	GRADE quality	Strength / implication
ATS adult CAP update, 11 November 2025	Clinical practice guideline update	Adults with CAP	For adult CAP outpatients who reach clinical stability, ATS suggests <5 days of antibiotics , minimum 3 days , rather than ≥ 5 days	Low	Conditional recommendation ATS
ATS adult CAP update, 11 November 2025	Clinical practice guideline update	Adult inpatients with non-severe CAP	Do not administer systemic corticosteroids in non-severe CAP	Low	Strong recommendation ATS
IDSA/PIDS pediatric CAP guideline update, 24 February 2026	Guideline using systematic reviews and GRADE	Children older than 3 months	Updated pediatric CAP guidance, especially diagnosis and management of parapneumonic effusion/empyema; systematic review performed for each question and GRADE used	Variable; report-assigned Moderate overall for guideline process	Recommendations are GRADE-based; apply to pediatric CAP and complicated pneumonia IDSA
IDSA/PIDS 2026 parapneumonic effusion/empyema update	Guideline update	Children/adolescents 3 months–18 years with pneumonia-associated empyema requiring drainage	Suggests chest tube placement with intrapleural fibrinolytic therapy as first-line management rather than surgical debridement such as VATS	Low to Moderate	Conditional based on available comparative evidence Infectious Disease Advisor
NICE 2025 pediatric CAP duration summary	Guideline summary	Children 3 months–11 years with non-severe CAP without underlying disease	Recommends 3-day antibiotic course for non-severe CAP; extend to 5 days for severe symptoms/signs	Moderate	Conditional to Strong , depending on severity BMJ Best Practice: pediatric CAP
WHO 2024 pediatric CAP duration as summarized by BMJ Best Practice	Guideline summary	Children under 5 years with non-severe CAP	WHO recommends 5 days , with 3 days in low-HIV-prevalence areas	Moderate	Strong BMJ Best Practice: pediatric CAP

Evidence / source	Design	Population	Key findings	GRADE quality	Strength / implication
Adult CAP PTC trial summarized in ATS/Stanford sources	Double-blind randomized placebo-controlled non-inferiority trial	Non-critical-care adult CAP	Discontinuing beta-lactam after 3 days was non-inferior in selected clinically stable patients; day-15 cure 78% placebo vs 68% continued beta-lactam; adverse events similar	Moderate	Supports shorter therapy in clinically stable adults Contagion Live, Stanford SHC pathway PDF
REGARD-VAP, 2024	Multicentre individually randomized open-label non-inferiority trial	Ventilator-associated pneumonia	Compared individualized short-course antibiotics with usual long-course treatment	Moderate	Supports antibiotic stewardship in selected VAP patients Stanford SHC pathway PDF
Rapid molecular/syndromic diagnostics: INHALE	Trial evidence summarized in review	HAP/VAP	Syndromic panels increased pathogen-directed therapy at 24 hours compared with standard culture	Moderate	Conditional use where available and linked to stewardship Pneumonia: Recent Updates on Diagnosis and Treatment
Rapid molecular/syndromic diagnostics: SARIPOC	Trial evidence summarized in review	Pneumonia requiring microbiologic testing	Antibiotic de-escalation 42% with syndromic panels vs 8% with culture; time to de-escalation 4.8 vs 46.5 hours	Moderate	Conditional use to improve de-escalation Pneumonia: Recent Updates on Diagnosis and Treatment
NICE 2025 corticosteroid evidence review	Evidence review	Adults and children with CAP/HAP	Corticosteroids may reduce inflammation but may increase infection risk; further research recommended, including pediatric populations	Low	No routine pediatric recommendation; cautious adult use only in selected contexts NICE PDF
NICE 2025 adult antibiotic summary	Guideline summary	Adults with non-severe pneumonia	Co-amoxiclav 500/125 mg three times daily for 5	Low to Moderate	Conditional; depends on local resistance and

Evidence / source	Design	Population	Key findings	GRADE quality	Strength / implication
			days then review for non-severe pneumonia without higher resistance risk		clinical severity Medscape/NICE summary
BMJ Best Practice adult CAP update, 18 September 2025	Evidence-based clinical summary	Adults with CAP	ATS 2025 suggests no empirical antibiotics for adult outpatients without comorbidities who have clinical/imaging CAP and test positive for a respiratory virus	Low	Conditional, requires reliable viral diagnosis and clinical judgment BMJ Best Practice PDF

2.3 Synthesis by Clinical Domain

2.3.1 Pediatric pneumonia: antibiotic duration

The WHO 2024 recommendation remains clinically central: **oral amoxicillin for 3 or 5 days** for children **2–59 months** with fast-breathing pneumonia only and no chest indrawing or danger signs [WHO guideline PDF](#). Newer pediatric guidance trends toward **shorter courses** in selected non-severe CAP. NICE 2025, summarized in BMJ Best Practice, recommends **3 days** for children 3 months to 11 years with non-severe CAP without underlying disease, extending to **5 days** for severe signs [BMJ Best Practice: pediatric CAP](#).

GRADE conclusion: Moderate-quality evidence supports short-course amoxicillin in selected uncomplicated pediatric pneumonia; recommendation remains **strong** where WHO criteria are met.

2.3.2 Pediatric complicated pneumonia

The 2026 IDSA/PIDS guideline update focuses on children older than 3 months and includes diagnosis and management of pneumonia with parapneumonic effusion [IDSA](#). For pneumonia-associated empyema requiring drainage, the update suggests chest tube plus intrapleural fibrinolytic therapy as first-line management rather than surgical debridement such as VATS [Infectious Disease Advisor](#).

GRADE conclusion: Low-to-moderate-quality evidence; conditional recommendation for drainage strategy, requiring local surgical, radiologic and pediatric expertise.

2.3.3 Adult pneumonia: WHO gap and non-WHO evidence

No current WHO adult pneumonia diagnosis/treatment guideline was identified in the supplied evidence. Adult management therefore relies on non-WHO sources such as ATS/IDSA, ATS 2025 update, NICE 2025 and BMJ Best Practice.

Key adult trends:

- Shorter antibiotic duration in clinically stable adult outpatients: **<5 days, minimum 3 days** [ATS](#).
- Avoid systemic corticosteroids in **non-severe adult inpatient CAP** [ATS](#).

- Use severity tools such as CURB-65 to guide site of care, with higher scores indicating need for hospitalization or ICU evaluation [BMJ Best Practice PDF](#).
- Consider withholding empirical antibiotics in selected adult outpatients without comorbidities who have CAP-compatible findings but a confirmed respiratory viral infection, according to ATS 2025 as summarized by BMJ Best Practice [BMJ Best Practice PDF](#).

GRADE conclusion: Adult short-course antibiotic and corticosteroid recommendations are generally **low-quality evidence** but guideline-supported; recommendations are conditional except for avoiding corticosteroids in non-severe inpatient CAP, which ATS labels **strong** despite low-quality evidence.

2.3.4 Rapid diagnostics and antimicrobial stewardship

Rapid molecular or syndromic panels may improve time to pathogen-directed therapy and de-escalation in HAP/VAP and selected pneumonia contexts. INHALE found more pathogen-directed therapy at 24 hours with syndromic panels, while SARIPOC reported antibiotic de-escalation in **42%** of syndromic-panel patients versus **8%** with culture, with time to de-escalation **4.8 hours** versus **46.5 hours** [Pneumonia: Recent Updates on Diagnosis and Treatment](#).

GRADE conclusion: Moderate-quality evidence for improved stewardship outcomes; clinical outcome and cost-effectiveness evidence remains less certain, so use should be **conditional** and linked to stewardship protocols.

2.4 Updated Recommendations Based on New Evidence

Table 3. WHO Recommendation Impact Assessment

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated actionable recommendation	GRADE quality	Strength
Children 2–59 months with fast-breathing pneumonia only should receive oral amoxicillin for 3 or 5 days	NICE 2025 and pediatric meta-analysis trends support short-course therapy in selected non-severe CAP	Continue WHO recommendation. Prefer 3 days in low-HIV-prevalence settings or where local protocols support short-course treatment; use 5 days where severe features, high-risk context or local policy indicates	Moderate	Strong
Antibiotics not routine for non-severe pneumonia with wheeze and no fever	Adult and pediatric stewardship trends support avoiding antibiotics when viral disease is likely	Continue to avoid routine antibiotics when bacterial pneumonia is unlikely; ensure reassessment and safety-netting	Low	Strong
Severe pneumonia requires referral/hospital care and parenteral antibiotics	No contrary evidence; complicated pneumonia guidance emphasizes structured escalation	Continue urgent referral/hospital management for danger signs, hypoxaemia or severe respiratory distress; use parenteral antibiotics per local resistance and WHO severe-pneumonia principles	Low to Moderate	Strong

Original WHO-based recommendation	New evidence impact through 29 April 2026	Updated actionable recommendation	GRADE quality	Strength
Use pulse oximetry if available and refer if SpO ₂ <90%	Pediatric and institutional CAP pathways continue to use <90% as a key hypoxaemia threshold	Maintain SpO₂ <90% as urgent oxygen/referral threshold in children; consider higher thresholds in high-risk patients according to local policy	Low to Moderate	Strong
Reassess outpatient pneumonia after 3 days	No evidence suggests longer interval is safer; short-course treatment increases need for clear safety-netting	Continue 3-day follow-up or earlier review if worsening, poor feeding, persistent fever, chest indrawing or danger signs	Low	Strong
No WHO adult pneumonia guideline identified	ATS/NICE/BMJ provide adult CAP updates	For adults, use national/local adult CAP guidelines; consider ATS 2025: shorter antibiotics in clinically stable outpatients and avoid steroids in non-severe inpatient CAP	Low to Moderate	Conditional , except steroids in non-severe inpatient CAP: Strong against

2.5 Evidence Gaps and Future Research Needs

1. WHO adult pneumonia guidance

- No current WHO adult pneumonia diagnosis/treatment/management guideline was identified in the supplied evidence.
- A WHO adult CAP guideline would be useful for low-resource and global implementation contexts.

2. Pediatric antibiotic duration

- More evidence is needed for children with comorbidities, malnutrition, high HIV prevalence, severe pneumonia and low-resource settings.
- WHO's 3-day versus 5-day approach should continue to be adapted to epidemiology, HIV prevalence and follow-up reliability.

3. Hypoxaemia thresholds

- SpO₂ <90% is operationally clear, but research is needed on thresholds for neonates, high-altitude settings, severe anaemia, malnutrition and chronic cardiopulmonary disease.

4. Rapid diagnostics

- Evidence supports faster de-escalation, but cost-effectiveness, mortality impact and implementation in low-resource settings remain uncertain.

5. Corticosteroids

- Adult severe CAP may benefit in selected contexts, but non-severe CAP should not routinely receive corticosteroids.

- Pediatric corticosteroid evidence remains insufficient; NICE identifies this as a research priority [NICE PDF](#).

6. Meaningful youth engagement

- WHO has produced a youth-engagement guideline-development case study for abortion care, not pneumonia [WHO webinar/event page](#).
- Future WHO pediatric pneumonia guideline updates should document whether children, adolescents, caregivers and youth representatives contributed to values, preferences, acceptability and implementation considerations.

2.6 Final Clinical Implementation Summary

Children

- **Assess severity first:** danger signs, chest indrawing, stridor, cyanosis/grunting, feeding ability and oxygen saturation.
- **Non-severe fast-breathing pneumonia, age 2–59 months:** oral amoxicillin for **3 or 5 days**.
- **Wheeze without fever:** avoid routine antibiotics; provide supportive care and reassessment.
- **SpO₂ <90%, danger signs or severe respiratory distress:** oxygen, urgent referral and hospital-level care.
- **Follow-up:** reassess after **3 days** or earlier if worsening.

Adults

- No WHO adult pneumonia guideline was identified.
- Use national adult CAP guidance; key post-2024 evidence supports:
 - Shorter antibiotics in clinically stable outpatients.
 - Avoiding corticosteroids in non-severe inpatient CAP.
 - Severity-guided site-of-care decisions.
 - Rapid diagnostics where results can drive antimicrobial stewardship.

Overall GRADE-based position

Clinical recommendation group	Overall evidence quality	Overall recommendation strength
WHO pediatric amoxicillin for uncomplicated fast-breathing pneumonia	Moderate	Strong
Avoid antibiotics for likely viral wheeze/no fever in children	Low	Strong
Oxygen/referral for SpO ₂ <90% or danger signs	Low to Moderate	Strong
Pediatric complicated pneumonia drainage strategy	Low to Moderate	Conditional
Adult shorter antibiotic courses in stable CAP	Low to Moderate	Conditional
Avoid systemic corticosteroids in non-severe adult inpatient CAP	Low	Strong against
Rapid diagnostics for stewardship in HAP/VAP or selected pneumonia	Moderate for de-escalation outcomes	Conditional